Levothyroxine

Tablets are strongly recommended as the formulation of choice (see below for administration advice). Liquid should only be used if tablets cannot be tolerated.

FORM 12.5 microgram and 25 microgram tablets

INDICATION Treatment of congenital hypothyroidism

DOSE RANGE

AGE	DOSE	FREQUENCY	ROUTE
Neonate with birth weight < 2.5 kg	Initially 10-15 micrograms/kg *then as directed by endocrine*	ONCE daily	Oral
Neonate with birth weight ≥ 2.5 kg and 1-6 months	Initially 50 micrograms *then as directed by endocrine*	ONCE daily	Oral

Doses should be rounded to nearest 12.5 microgram.

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS
- See Summary of Product Characteristics and most recent edition of BNF for Children

FURTHER INFORMATION

- Tablets should be crushed and mixed with a small volume (5ml or less) of sterile water for administration. Formula or expressed breast milk may be used if water is not tolerated
- Levothyroxine may cause tachycardia and can increase blood glucose levels.
- Avoid giving other medicines at the same time
- Tablets should be crushed and dissolved in a maximum of 5ml sterile water. If not tolerated, alternatively use a maximum of 5ml breast milk or formula. The reconstitution fluid should stay the same to minimise bioavailability issues.

LICENSED STATUS Licensed for use in all ages

APPLICABLE POLICIES West of Scotland Neonatal Guidelines:

Scottish Paediatric Endocrine Group Guidelines

Consult local policy if applicable

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Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Group. Information is correct at the time of publication and as per local practice agreement.
For further advice contact your clinical pharmacist or pharmacy department.